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/ **Practice Exam - 1**

Correct Answer

Next Q

Practice Exam - 1

Question 69 of 117



A 59-year-old man with chronic heart failure and reduced ejection fraction, non-obstructive CAD, systemic hypertension, T2DM and chronic kidney disease stage 3, was hospitalized for an episode of acute decompensated heart failure. He recovered well and you are seeing him for follow up in the office after discharge. He has improved since discharge but remains congested on examination. Current medications include: ASA 81 mg/d, Losartan HCT 100/25 mg/d, Rosuvastatin 20 mg/d, Torsemide 20 mg/d, rosiglitazone 4 mg/d and metformin 2,000 mg/d.

Physical exam revealed an oriented patient with BP 117/80 mmHg and HR 80bpm, BMI 33 kg/m², JVP at 12cmH₂O, basilar crackles, S₃ gallop and 2+ pitting edema of lower extremities. Echocardiogram showed LVEF of 40%, grade 2 diastolic dysfunction, 1+MR and systolic, and estimated PAP of 45 mmHg. CXR showed pulmonary vascular congestion.

Laboratory data: A1C: 8.7%; eGFR: 52 ml/min/1.73m²; fasting glucose 176 mg/dl; Hb 11.4g/dl; LDL cholesterol 78 mg/dl; triglycerides 256 mg/dl.

Which modification to this patient's glucose-lowering regimen is most appropriate to improve glycemic control while also reducing heart failure morbidity?

- A. Add dapagliflozin 10 mg/d to rosiglitazone and metformin
- ✓ B. Add empagliflozin 10 mg/d to metformin 2,000 mg/d and discontinue rosiglitazone
- C. Discontinue to rosiglitazone and initiate insulin
- D. Lower rosuvastatin dose and initiate insulin
- E. Discontinue Metformin and saxagliptin

Commentary

References

Testing Point: Exam Takers should know that SGLT2i reduce HF re-admission rate and decrease CV mortality in diabetic patients.

Correct Answer: B. Add empagliflozin 10 mg/d to metformin 2,000 mg/d and discontinue rosiglitazone

Rationale: This patient has heart failure with reduced or mildly reduced ejection fraction (LVEF 40%), type 2 diabetes mellitus, chronic kidney disease, and persistent congestion following recent hospitalization for acute decompensated heart failure. In patients with heart failure and T2DM, current guideline-directed therapy favors use of SGLT2 inhibitors because these agents reduce heart failure hospitalization, cardiovascular mortality, and progression of chronic kidney disease, independent of baseline hemoglobin A1c.

Empagliflozin is an appropriate addition given this patient's renal function (eGFR 52 mL/min/1.73m²) and established heart failure. In contrast, rosiglitazone, a thiazolidinedione, should be discontinued because thiazolidinediones promote sodium and fluid retention and are associated with an increased risk of heart failure exacerbation and hospitalization.

Answer A: Although dapagliflozin is also an appropriate SGLT2 inhibitor, continuing rosiglitazone is not recommended because of its adverse effects in patients with symptomatic heart failure.

Answers C and D: Insulin therapy may improve glycemic control but does not confer the cardiovascular or heart failure benefits demonstrated with SGLT2 inhibitors and is not the preferred next step in this patient.

Answer E: Metformin remains appropriate in stable heart failure and chronic kidney disease with eGFR >30 mL/min/1.73m². Additionally, saxagliptin has been associated with increased risk of heart failure hospitalization and should generally be avoided in patients with heart failure.

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